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Name: امينه محمود حسين - National ID: 24503192101088	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



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04/01/2026

MDT FORM

Name: / Mahmoud /	MRN: 3725
Age: 81 Y	ID: 24503192101088
Menopausal Sta: post	Phone: 01122572218
NRS SCORE:	Marital Status:
BMI:	Family History: no
CO Morbidity: nad	Reason Of MDT:

Clinical Data:

BX: infiltrating ductal carcinoma
ER: 8/8 **PR:** 8/8 **HER2:** +1 **KI67:** 20

Sonomammography:

It uoq no clustre of mirocalcification birads 5 and today for last month

Mets Workup:

ct cap free

History:

patient started al for 6m as neoad treatment

Recommendations:

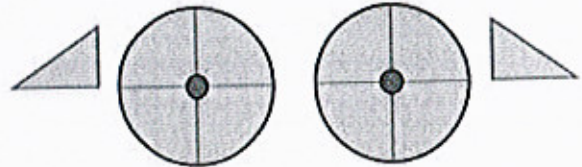
for surgical consult

MDT Outcome & Plan

In view of having initially cT4b N+ disease....for Lt MRM

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SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ شيماء لاشين	د/ امل عيسى	د/ ملى زكى			

14/2026
Mona Tay 14.2026

New MDT FORM 14/9/2025G

Name	اسم المريض	MRN	٢٢٦٩/٢٢٥٠
Age	80	ID	٢٢٥٠٢١٩٢١٠٠٨٨
Menopausal status	Post	Phone	٠١١٢٢٧٥٢٢١٨
NRS SCORE		Marital status	٠١١٢٢١٧٥٠٢٨
BMI		Family history	- FH
CO morbidity	NAD	REASON OF MDT	New

Clinical Data

BX: infiltrating Duct of GII

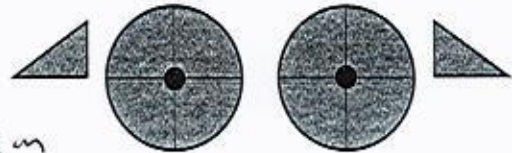
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ER: 8 PR: 8 HER 2: +1 Ki67: 20%

Sonomammography: 1st UOQ no clusters
of microcalcification 3-4 o'clock 4.5x4cm
BIRADS 5

Mets Workup: CT CAP => free



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Recommendations:

1. New AD AI X 6ms
of Re one

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



Women's Health Outreach Program

Name: Amina Mahmoud Hussin

Date: 23-7-2025

Full field Bilateral Digital Mammography & ultrasonography

Clinical summary: Left breast ulcerating lesion for 3 yrs duration.

Detailed Findings

Technique

Craniocaudal & mediolateral oblique views

High resolution ultrasound

Right breast

- ACR density A.
- No masses, asymmetry or distortion.
- Ultrasound demonstrated No solid or cystic changes.
- Normal skin/nipple/areolar complex.
- Preserved axillary LNs largest size, cortical thickness.

Left breast

- ACR density A
- Left UOQ outer zone large irregular shape speculated dense lesion No suspicious clusters of microcalcifications.
- Ultrasound demonstrated Large irregular shaped speculated lesion seen at 3-4 o'clock measuring 4.5x 4 cm with intraductal extension reaching the nipple and reaching skin with clinical skin ulceration and tethering.
- Retro areolar dilated ducts with echogenic contents .
- Normal skin/nipple/areolar complex.
- Few globular shaped pathological axillary nodes .

Conclusion:

-Left breast: Locally advanced UOQ highly suspicious malignant lesion with intra-ductal extension reaching the nipple and pathological axillary nodes (BIRADS 5).

-Right breast: Normal study (BIRADS 1).

Recommendation ; Histopathological correlation

Best regards,

Dr. Heba Ahmed, MD

مركز أورام دار السلام (هرمل سابقاً) - ٩٨٧ شارع كورنيش النيل - الملك الصالح
الدور السادس - مقر المبادرة الرئاسية لدعم صحة المرأة



ImmunohistoChemistry report

Report Number: m1162

Female	النوع	امينة محمود حسين	اسم المريض
القاهرة	العنوان	80 Y	السن
29/07/2025 13:55	تاريخ استلام العينة		تاريخ اخذ العينة
04/08/2025 09:44	تاريخ التقرير	مركز صحة المرأة	طالب التحليل
		مركز صحة المرأة	عناية

Primary Sample :

Paraffin Block

Test :

ER & PR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor(PR) (IE2)Rabbit monoclonal primary antibody.

Sample recieved was treated by: - VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX / ULTRA .

Sample recieved was treated by: - CONFIRM anti Ki67 (30-9) Rabbeit monoclonal primary antibody.

Result :

ER: strong nuclear staining of > 60 % of tumour cells (core biopsy)
PR: strong nuclear staining of > 60 % of tumour cells (core biopsy)
HER2/neu protein over expression : faint discontinuous membrane staining of tumor cells (core biopsy).
Ki67 :positive nuclear staining of about 20 % of tumour cells (core biopsy)

Referral Lab :

Comment :

Conclusion :

ER: POSITIVE (SCORE 8 / 8)
PR: POSITIVE (SCORE 8 / 8)
HER2/NEU PROTEIN OVER EXPRESSION : NEGATIVE (SCORE 1+)
Ki67: THE PROLIFERATION INDEX OF THE TUMOUR IS ABOUT 20 % .

Pathologist: د/تريزا ماهر

Lab Director: د/تريزا ماهر





Histopathology report

Report Number: 203

Female	: النوع	امينة محمود حسين	: اسم المريض
القاهرة	: العنوان	80 Y	: السن
28/07/2025 09:39	: تاريخ استلام العينة		: تاريخ اخذ العينة
30/07/2025 09:59	: تاريخ التقرير	مركز صحة المرأة	: طالب التحليل
		مركز صحة المرأة	: عناية

Primary sample :

Left breast core biopsy

comment :

Gross examination :

cores of greyish white tissue (T.I)

Microscopic examination :

Section examined revealed fibrocollagenous tissue infiltrated by malignant growth displaying the morphology of infiltrating duct carcinoma

Referral Lab :

Comment :

Diagnosis :

LEFT BREAST CORE BIOPSY: THE PICTURE IS COMPATIBLE WITH INFILTRATING DUCT CARCINOMA , GRADE II

Pathologist: DrJehan Aziz

Lab Director: د/تريزا ماهر

New MDT FORM 14/9/2025G

Name	امينة محمد حسين	MRN	٢٢٦٩/٢٠٥٥
Age	80	ID	٢٤٥٠٢١٩٢١٠٠٨١
Menopausal status	Post	Phone	٠١١٢٢٧٥٢٢١١
NRS SCORE		Marital status	٠١١٢٤١٧٥٠٢١
BMI		Family history	- FH
CO morbidity	NAD	REASON OF MDT	New

Clinical Data

BX: infiltrating Duct of GD

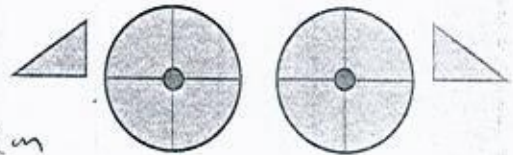
ER: 8 PR: 8 HER 2: +1 Ki67: 20%

Sonomammography: Lt UOQ no clusters
of microcalcification 3-4 o'clock 4.5x4cm
BIRADS 5

Mets Workup: CT CAP => free

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Recommendations:

1. New AD AI X 6ms
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MDT OUTCOME & PLAN

SIGNATURE

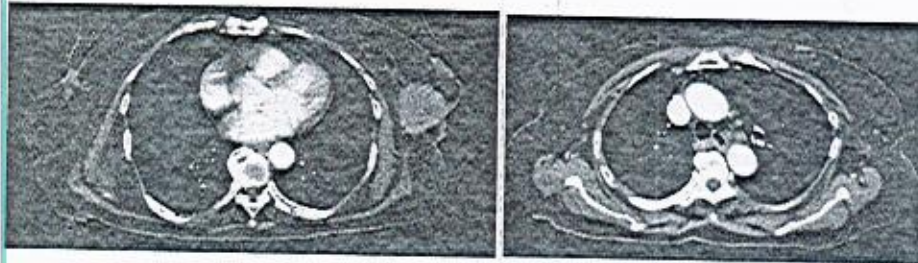
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name: Ameena Mahmoud Hussein Mohamed	Date : 24/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 80 Year(s)
Medical ID:	Pat. ID: 111103

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

Clinical History:

- Known case of left breast cancer, no available previous films for comparison.



Findings:

- The left breast lower outer quadrant shows a large speculated soft tissue mass lesion measuring about 4x4.5 cm seen adherent to the overlying skin.
- Few left axillary lymph nodes are noted, the largest seen measuring 16x13 mm.
- Few right axillary lymph nodes are noted, the largest seen measuring 10x7mm.
- Bilateral few scattered atelectatic bands are noted.
- No sizable nodules, masses or consolidative patches.
- No sizable hilar or mediastinal lymph nodes.
- Patent tracheo-bronchial tree.
- Normal CT appearance of the mediastinal vessels.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.
- The lower neck cuts revealed bulky heterogeneous right thyroid gland for ultrasound correlation.
- NB. Bilateral fibroelastoma dorsi is noted.

OPINION:

- Left breast LOQ speculated soft tissue mass lesion for sonomammography correlation.
- Few left axillary lymph nodes.
- Few right axillary lymph nodes.

Marina Essam, MSc
Dr.Noura Nabil MD

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS: -

Suboptimal study due to motion artifact

Clinical History:

- Known case of left breast cancer, no available previous films for comparison.



Findings:

- The liver is enlarged (17 cm) with homogenous CT texture. No sizable focal lesions.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Evidence of cholecystectomy surgical clips for surgical history correlation.
- Average size of the spleen (8.5 cm) with no focal lesions seen.
- Normal appearance of the pancreas and both suprarenal glands.
- The right kidney shows a large pelvic branching stone (staghorn stone) with mild pelvicalyceal system dilatation. A large lower polar cortical cyst is also noted measuring about 6x6.5 cm.
- Normal size, parenchymal thickness and density of both kidneys with no stones or back-pressure changes.
- Inadequate filling of the urinary bladder yet with no sizable stones or masses.
- Right sacral ala sclerotic lesion is noted for follow up.
- Spondylo-degenerative changes are noted.

Opinion:

- *Hepatomegaly for clinical and lab correlation*
- *Right renal pelvic stone with mild hydronephrotic changes.*
- *Right sacral ala sclerotic lesion for follow up.*

Marina Essam, MSc
Dr.Noura Nabil,MD

Signature Noura Nabil
28/08/2025 1:22 AM